COUN 5553 — Quiz 1 Practice Questions (Set 2)

Chapters 1–8 | 30 Questions | Multiple Choice & True/False

Chapter 1: Introduction

1. Legal standards, ethical standards, and best practice represent different levels of professional behavior. Which of the following correctly orders them from the *minimum* to the *ideal*?

- A) Ethical standards → Legal standards → Best practice
- B) Legal standards → Ethical standards → Best practice
- C) Best practice → Ethical standards → Legal standards
- D) Legal standards → Best practice → Ethical standards

2. According to your textbook, the term *morality* refers to:

- A) Professional behaviors expected of counselors
- B) Personal beliefs and how those beliefs affect your conduct
- C) Legal standards imposed by state legislatures
- D) The code of ethics created by a professional organization

3. In the eight-step ethical decision-making model presented in class, Step 6 is:

- A) Identify the problem or dilemma
- B) Make a decision and act on your choice
- C) Consult with colleagues or experts
- D) Review the relevant ethics codes

4. A counselor who acts ethically because the rules say so — regardless of the outcome — is operating from which ethical theory?

- A) Utilitarianism
- B) Relativism
- C) Absolutism
- D) Egoism

Chapter 2: Professional Identity of Counselors

5. Which of the following is NOT one of the four philosophical beliefs underlying the counseling profession?

- A) The wellness model of mental health
- B) A developmental perspective
- C) The medical model of diagnosing and treating illness
- D) Empowerment of clients

6. The first counselor licensure law in the United States was passed in Virginia in:

- A) 1953
- B) 1961
- C) 1976
- D) 1982

7. The National Board for Certified Counselors (NBCC) was created in:

- A) 1961
- B) 1976
- C) 1982
- D) 2014

8. According to the ACA Code of Ethics (2014, C.4.d.), counselors who hold a master's degree in counseling and a doctoral degree in an unrelated field should:

- A) Use "Dr." freely in all professional contexts
- B) Not imply doctoral-level competence in counseling when their doctorate is not in counseling or a related field
- C) List only their doctoral degree on business cards
- D) Avoid mentioning any degree credentials

Chapter 3: Multiculturalism, Social Justice, Advocacy, and Values

9. In the ADDRESSING model (Hays, 2002), the letters stand for dimensions counselors should consider in assessment and treatment. The "D" in ADDRESSING stands for:

- A) Diagnosis
- B) Developmental and acquired Disability
- C) Diversity
- D) Discrimination

10. According to James Marcia's theory of identity development (discussed in Class 2), identities that are defined externally rather than internally are called:

- A) Moratoriums
- B) Foreclosures
- C) Achieved identities
- D) Diffused identities

11. Social justice and advocacy are both ACA competencies. The key distinction between them is:

- A) Social justice involves action; advocacy involves promoting access
- B) Social justice promotes access to services for marginalized people; advocacy is the commitment to action to promote change
- C) They are the same concept with different names
- D) Social justice applies only to counselor educators; advocacy applies to all counselors

12. According to Class 2 slides, the concept of "double vision" (Volf, 2019; Eriksson & Abernethy, 2014) refers to:

- A) Seeing through two theoretical lenses at once
- B) Seeing experiences through another's perspective empathically without leaving behind one's own views and values
- C) Using two different assessment tools for each client
- D) Viewing clients through both the medical and wellness models

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Chapter 4: Client Welfare and Informed Consent

13. When a counselor's unresolved personal conflicts (e.g., a need to have all the answers, a desire to take away all pain from clients) become entangled in the counseling relationship, this is called:

- A) Transference
- B) Countertransference
- C) Projection
- D) Displacement

14. Which of the following is TRUE about informed consent?

- A) It only needs to be discussed once at intake
- B) It is purely an ethical obligation, not a legal one
- C) It must include information about the role of technology and a backup plan if tech fails
- D) It is only required for clients over 18

15. According to the ACA Code of Ethics (2014, C.7.a.), specialized counseling techniques should be grounded in:

- A) The counselor's personal preference
- B) Empirical research
- C) The client's insurance requirements
- D) Whatever the supervisor recommends

16. A *fiduciary* relationship in counseling means:

- A) The client pays the counselor a fee for services
- B) Both the counselor and client share equal responsibility
- C) The counselor acts in the client's benefit — the client is not there for the counselor's benefit
- D) The counselor has a legal duty to report all client information to the court

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Chapter 5: Confidentiality and Privileged Communication

17. Privacy, confidentiality, and privileged communication are related but distinct concepts. Which of the following correctly describes the relationship?

- A) Privacy is the narrowest term; privileged communication is the broadest
- B) Privacy is the broadest term; confidentiality is an explicit promise not to reveal information; privileged communication is a legal protection in court
- C) All three terms mean the same thing
- D) Privileged communication is an ethical concept; confidentiality is a legal concept

18. In Washington State, a statutory privilege belongs to:

- A) The counselor
- B) The client
- C) The court
- D) The counselor's supervisor

19. According to the Class 3 slides, the *Bradley* case established the:

- A) Duty to warn the potential victim
- B) Duty not to negligently release a dangerous client
- C) Extension of duty to warn to anyone near the intended victim
- D) Protection of communications under federal law

20. Under Washington State law (RCW 71.34.650), parents may seek outpatient mental health services for an adolescent (13+) without the adolescent's consent for up to:

- A) 6 sessions in a 2-month period

- B) 12 sessions in a 3-month period
- C) 24 sessions in a 6-month period
- D) An unlimited number of sessions

Chapter 6: Records and Subpoenas

21. According to the ACA Code of Ethics (B.6.e.), competent clients should have:

- A) No access to their records under any circumstances
- B) Reasonable access to their counseling records
- C) Full unrestricted access to all records including those from outside professionals
- D) Access only if ordered by a court

22. Under HIPAA, counselors must do all of the following EXCEPT:

- A) Give clients a clear written explanation of how their health care information is used, kept, and disclosed
- B) Give clients access to their records (unless it would cause them harm)
- C) Automatically share all records with family members without the client's knowledge
- D) Keep a written history of disclosures of client information, available to clients

23. In the SOAP note format, the "A" stands for:

- A) Action
- B) Assessment (making sense of the Subjective and Objective data)
- C) Appointment
- D) Advocacy

Chapter 7: Competence, Assessment, and Diagnosis

24. When the ACA Code of Ethics says counselors should practice "within the boundaries of their competence," the most salient moral principle at stake is:

- A) Autonomy
- B) Fidelity
- C) Nonmaleficence ("do no harm")
- D) Veracity

25. *Vicarious traumatization* is listed in the textbook as a risk factor for:

- A) Client noncompliance
- B) Counselor impairment
- C) Malpractice lawsuits
- D) Diagnostic errors

26. In Washington State, the requirements for LMHC licensure include passing the NCE or NCMHCE exam AND completing:

- A) 1,000 hours of supervised experience
- B) 3,000 hours (including 1,200 direct hours and 100 supervision hours) OR 3 years of full-time work
- C) 5,000 hours of post-graduate experience
- D) 600 hours of internship only

27. According to ACA Code of Ethics sections C.2.g and F.5.b, if counselors determine they are impaired, they must:

- A) Continue practicing but add extra supervision
- B) Limit, suspend, or terminate their professional responsibilities and seek assistance
- C) Immediately retire from the profession
- D) Refer all clients to their supervisor without telling them why

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Chapter 8: Malpractice and Resolving Legal and Ethical Challenges

28. The next leading cause of malpractice lawsuits against counselors (after sexual relationships with clients) involves situations in which:

- A) Counselors break confidentiality
- B) Counselors provide an incorrect diagnosis
- C) Clients attempt or complete suicide
- D) Counselors fail to keep adequate records

29. According to class materials, counselors can be held liable for both *overreacting* and *underreacting* when a client presents with suicide risk.

- **True** or **False**?

30. If an angry client threatens to sue you, which of the following is the recommended FIRST step?

- A) Immediately contact your liability insurance company
- B) Respond directly to the person making the threat — calm the person down and listen to their concerns
- C) Refuse to speak with them and refer them to your attorney
- D) Admit wrongdoing to de-escalate the situation

ANSWER KEY

Chapter 1

1. **B** — Legal standards are the minimum behavior society will tolerate; ethical standards are the minimum counselors will tolerate from each other; best practice is the ideal. (Class 1 slides p. 47)
2. **B** — Morality refers to personal beliefs and how they affect conduct, whereas ethics refers to professional behaviors. (Ch. 1, p. 3; textbook summary p. 26)
3. **C** — The eight-step model: (1) Identify the problem, (2) Involve client, (3) Review ethics codes, (4) Consider principles and virtues, (5) Tune into feelings, (6) **Consult with colleagues or experts**, (7) Consider context, (8) Make a decision. (Class 1 slides pp. 69-70; Class 2 slides pp. 50-51)
4. **C** — Absolutism holds that moral principles are fixed and must be followed regardless of the outcome or context. This differs from deontology (rule-based, Kant) but shares the idea that certain actions are right or wrong in themselves. (Ch. 1, p. 10)

Chapter 2

5. **C** — The four beliefs are: (1) wellness model, (2) developmental perspective, (3) prevention and early intervention, and (4) empowerment of clients. The medical model is NOT one of them — counseling explicitly distinguishes itself from the medical model. (Ch. 2, pp. 30-34; Class 1 slides pp. 29-32)

6. **C** — Virginia passed the first counselor licensure law in 1976. (Ch. 2, p. 45; Class 1 slides p. 39)
7. **C** — NBCC was created in 1982 as the national agency to certify counselors. (Ch. 2, p. 39)
8. **B** — ACA Code C.4.d. states counselors should not imply doctoral-level competence in counseling when their doctorate is not in counseling or a closely related field. (Ch. 2, pp. 41-42)

Chapter 3

9. **B** — ADDRESSING: Age, Developmental and acquired Disability, Religion/spirituality, Ethnicity, Socioeconomic status, Sexual orientation, Indigenous status, Nationality, Gender. (Class 2 slides p. 15; Hays, 2002)
10. **B** — Marcia's theory states that identities defined externally (by others) rather than internally are foreclosures. (Class 2 slides p. 34)
11. **B** — Social justice = promoting access for all people to mental health services, especially marginalized populations. Advocacy = commitment to social justice through ACTION to promote change. (Class 2 slides p. 21)
12. **B** — "Double vision" (Eriksson & Abernethy, 2014, drawing on Volf) means seeing through another's perspective empathically without abandoning one's own views — requiring a high level of differentiation. (Class 2 slides p. 49)

Chapter 4

13. **B** — Countertransference is when the counselor's own feelings, attitudes, and unresolved issues become entangled in the therapeutic relationship. Transference is when the *client* projects past feelings onto the counselor. (Class 3 slides pp. 8-11)
14. **C** — Informed consent must include the role of technology and a backup plan if tech fails (item 5 on the 10-element list). It is both a legal AND ethical obligation, and it is an ongoing process — not a one-time event. (Class 2 slides p. 70; Class 3 slides p. 26)
15. **B** — ACA C.7.a. requires that specialized techniques be grounded in empirical research. (Class 2 slides p. 60; Class 3 slides p. 15)
16. **C** — Fiduciary means the counselor acts in the client's benefit. "Clients are not there for your benefit, you are there for them." (Class 2 slides p. 57; Class 3 slides p. 7)

Chapter 5

17. **B** — Privacy is the broadest term (individual's right to decide what is shared). Confidentiality is an explicit promise between counselor and client. Privileged communication is a legal protection in court proceedings. (Class 3 slides p. 37)
18. **B** — Privilege belongs to the client, not the counselor. The client asserts or waives the privilege. If the client cannot be located, the counselor should assert it on their behalf. (Class 3 slides pp. 69, 71)
19. **B** — The Bradley case established the duty not to negligently release a dangerous client. (Class 3 slides p. 52)
20. **B** — Under RCW 71.34.650, parents may seek outpatient mental health services for adolescents 13+ for up to 12 sessions in a 3-month period without the adolescent's consent. (Class 3 slides p. 45)

Chapter 6

21. **B** — ACA B.6.e. states that competent clients should have reasonable access to their counseling records. (Class 4 slides p. 4)
22. **C** — HIPAA does NOT allow automatic sharing with family members without the client's knowledge. However, if clients are informed and given an opportunity to orally object, counselors may share information with family members assisting in the client's care. (Class 4 slides pp. 15-16)

23. **B** — SOAP: Subjective (client reported), Objective (counselor observed), Assessment (making sense of S and O), Plan (action steps). (Class 4 slides pp. 6-7)

Chapter 7

24. **C** — Nonmaleficence ("do no harm") is the most salient moral principle related to competence. Practicing outside one's competence risks harming the client. (Class 4 slides p. 21; Ch. 7, p. 161)
25. **B** — Vicarious traumatization is listed alongside environmental factors and preexisting conditions as risk factors for counselor impairment. (Class 4 slides p. 33; Ch. 7, pp. 173-176)
26. **B** — WA LMHC requires passing the NCE/NCMHCE AND 3,000 hours (1,200 direct, 100 supervision) or 3 years of full-time counseling work. (Class 4 slides p. 25)
27. **B** — ACA C.2.g and F.5.b state that impaired counselors must limit, suspend, or terminate their professional responsibilities and seek assistance. (Class 4 slides p. 36)

Chapter 8

28. **C** — The next leading cause of malpractice lawsuits (after sexual relationships) involves situations where clients attempt or complete suicide. (Class 4 slides p. 53; Ch. 8, p. 199)
29. **True** — Courts can hold counselors accountable for both overreacting (e.g., unnecessary involuntary commitment) and underreacting (e.g., failing to take protective steps). (Class 4 slides p. 58; Ch. 8)
30. **B** — The recommended first step is to respond directly to the person — calm them down and listen to their concerns. Do NOT admit wrongdoing. (Class 4 slides p. 65)